

TRAUMA

T-03 Head and Spinal Trauma BLS

PRIORITIES:

- ABCs
- Identify airway compromise/obstruction, respiratory insufficiency/arrest, active bleeding, shock, altered mental status and initiate appropriate management
- Assure an advanced life support response

HEAD TRAUMA

1. Manage airway using the appropriate adjunct. If spinal injury is suspected use the jaw-thrust technique.
2. Support ventilation.
3. Perform Spinal Motion Restriction (SMR) as defined in Solano County EMS Policy 6611.
4. **CONSIDERATIONS**
 - Control face and head bleeding with direct pressure. Utilize extreme caution with potential skull fractures.
 - Check the oropharynx for teeth and dentures.
 - Frequent airway suctioning is needed to prevent aspiration of blood, etc.
 - Avoid applying direct pressure to an injured eye. Do not attempt to replace the partially torn globe – stabilize it in place with a saline soaked gauze.
 - **DO NOT** assume alcohol intoxication as primary cause. All persons with altered level of consciousness associated with trauma require treatment and transport to a medical facility where the patient can receive a complete medical examination.

SPINAL TRAUMA

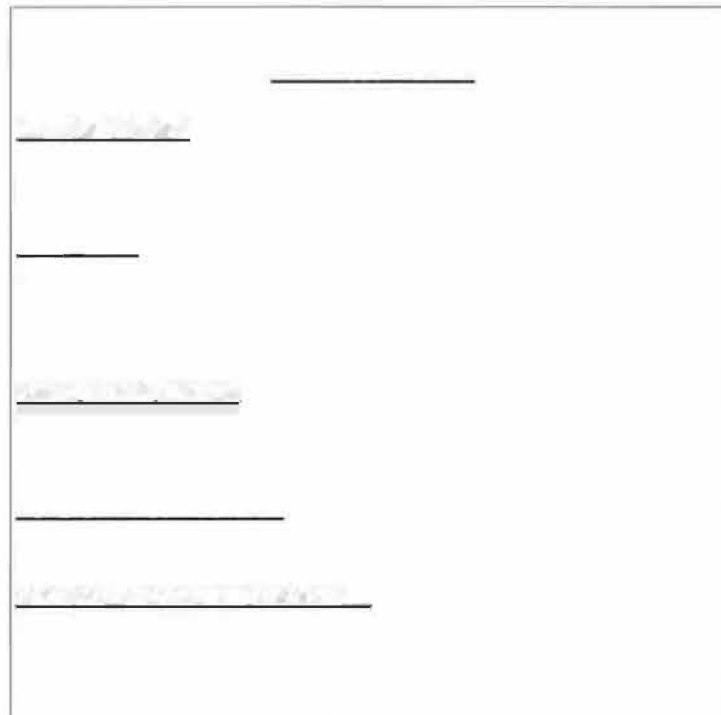
For treatment of spinal trauma, please refer to Solano County EMS Policy 6611, Spinal Motion restriction, for SMR procedures.

Trauma

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- Stabilize airway
- Spinal Motion Restriction per Policy 6611
- Hyperventilate patients with signs of cerebral herniation
- Assess for and remove all foreign body airway obstructions. If the foreign body cannot be removed, perform a needle cricothyrotomy per Protocol S-4.
- Control external hemorrhage using direct pressure or hemostatic dressings. Do not use a tourniquet on head or neck hemorrhaging.



DISRUPTED COMMUNICATIONS

In the event of a "disrupted communications" situation where a base hospital physician CANNOT be contacted for orders, Solano County Paramedics MAY NOT utilize the portions of this protocol requiring base physician orders AND must transport to the closest receiving facility.